

Radiotherapy for womb cancer

Radiotherapy uses high-energy rays called radiation to destroy cancer cells. You may have radiotherapy as a treatment for womb cancer.

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What is radiotherapy?

Radiotherapy uses high-energy rays called radiation to treat cancer. It destroys cancer cells in the area where the radiotherapy is given.

There are 2 ways of having radiotherapy for [womb cancer](#):

- External beam radiotherapy is given from outside the body (externally) using a radiotherapy machine.
- Internal radiotherapy is when a radioactive material is placed inside the body. This is called brachytherapy.

Sometimes radiotherapy is given after [chemotherapy](#). The chemotherapy makes the cancer cells more sensitive to radiotherapy.

When radiotherapy is used for womb cancer

You may have radiotherapy for womb cancer:

- after [surgery](#), to reduce the risk of the cancer coming back (adjuvant radiotherapy)
- instead of surgery, if a general anaesthetic or surgery is not suitable

- to try to cure womb cancer that has come back after surgery (recurrent cancer)
- to treat womb cancer that was not completely removed with surgery
- to help control symptoms, or if womb cancer has spread to other parts of the body (palliative radiotherapy).

If the cancer has spread to only one place, and it is small, it may be possible to have a type of targeted radiotherapy called [stereotactic radiotherapy](#). This is only suitable for a small number of people. Your doctor will explain more about this if it is suitable for you.

Radiotherapy after surgery

Your cancer specialist may talk to you about having radiotherapy after surgery. This is to reduce the risk of the cancer coming back. This is called adjuvant treatment. Your cancer doctor will look at the [type](#), [stage](#), [grade](#), genetic changes and risk group of your cancer to decide if adjuvant treatment might be suitable for you.

You may not need adjuvant treatment if you had a lower risk cancer. But your doctor may talk to you about having adjuvant treatment if you have a higher risk of the cancer coming back in the future.

Radiotherapy to control symptoms

If the cancer has spread, you may have external beam radiotherapy to shrink the cancer or to control the symptoms. This is called [palliative treatment](#).

Macmillan is here to support you. If you would like to talk, you can:

- call the Macmillan Support Line on [0808 808 00 00](tel:0808 808 00 00).
- [chat](#) to our specialists online.
- visit our [womb cancer forum](#) to talk with people who have been affected by womb cancer, share your experience, and ask an expert your questions.

External beam radiotherapy

[External beam radiotherapy](#) is when a radiotherapy machine aims high-energy rays at the area of the body being treated.

Planning external beam radiotherapy

Before you start your treatment, it needs to be planned. This is to make sure that the radiotherapy is aimed directly at the cancer. This will also mean it causes the least amount of damage to the surrounding area. Your [cancer team](#) will plan your radiotherapy carefully.

You will have a hospital appointment to plan your treatment. You will usually have a [CT scan](#) of the area to be treated. During the planning appointment, you may have markings made on your skin. These help the radiographers position you for each treatment.

Treatment sessions

External beam radiotherapy treatment is normally given as a number of short, daily treatments in a radiotherapy department. These are called treatment sessions or fractions. The radiotherapy machine looks like a large x-ray machine or CT scanner.

You have treatment in the radiotherapy department at the hospital. You have it as an outpatient once a day from Monday to Friday, with a rest at the weekend. A full course may last up to 5 to 6 weeks. External beam radiotherapy is painless. It will not make you radioactive. It is safe for you to be around other people, including children and pregnant women throughout treatment.

Usually, each external beam radiotherapy appointment takes about 10 to 30 minutes. But you may be in the department for longer. The treatment itself usually only takes a few minutes. Most of the appointment is spent getting you into the correct position and checking your details.

During treatment, the radiotherapy machine will move around you, but it does not touch you. Once you are comfortable in the correct position, they will ask you to keep as still as possible. They will leave the room for a few minutes while you have your treatment. There will be a camera so they can see you and hear you if you need to talk to them.

Side effects of external beam radiotherapy

Side effects do not usually happen straight away. They may develop during treatment or in the days or weeks after treatment finishes. Sometimes side effects get worse for a time during and after you have finished radiotherapy.

Your team will explain what to expect. Always tell them if you have side effects during or after radiotherapy. They can give advice and support to help you cope.

Radiotherapy to the womb and surrounding areas it is known as pelvic radiotherapy. We have more information about the side effects in our information about [pelvic radiotherapy](#).

Tiredness

Radiotherapy often makes people feel tired. Tiredness (fatigue) may get worse as treatment goes on. If you are having radiotherapy alongside other treatments, such as [surgery](#) or [chemotherapy](#), you may feel more tired. But there are things you can do to help, such as:

- get plenty of rest
- do some gentle exercise, such as going for short walks
- eat a healthy diet and drink plenty of fluids
- ask for help with everyday tasks, if you have friends or family members who can support you.

After treatment finishes, you may continue to feel tired for weeks or months. If it does not get better, tell your cancer doctor or specialist nurse.

We have more information about [coping with tiredness \(fatigue\)](#).

Bowel changes

[Diarrhoea](#) is the most common side effect. It usually starts during radiotherapy or after 2 weeks of treatment. If you have diarrhoea, your radiotherapy team may give you tablets to help reduce it. Try to drink at least 2 to 3 litres (8 to 12 glasses) of fluids each day, unless your doctor or nurse tells you something different.

If you have any bowel (poo) incontinence, tell your radiotherapy team. They will give you advice about coping with incontinence and looking after your skin.

It is important to tell your radiotherapy team about any bowel side effects. They may suggest making some changes to your diet during radiotherapy. Or they may suggest treatments to help. Follow their advice carefully. Tell them if your side effects get worse.

We have more information about the [bowel side effects of pelvic radiotherapy](#).

Bladder changes

Pelvic radiotherapy can [irritate the lining of the bladder](#). You may need to pee (urinate) more often. You may find it hard to wait when you need to go, or you may leak a bit of urine. You may notice a burning feeling when you pass urine (pee), which feels like having a urine infection. You may also notice blood in your urine (haematuria).

Always tell your radiotherapy team if you have any side effects during or after your treatment. They can give you advice and may recommend medicines to help you feel more comfortable. They may also ask you for a urine sample to check if you have an infection.

We have more information about the [bladder side effects of pelvic radiotherapy](#) and [coping with bladder changes](#).

Vaginal changes

Pelvic radiotherapy can affect the vagina. It may cause changes such as:

- skin reactions
- dryness
- bleeding
- narrowing of the vagina.

Vaginal changes can be uncomfortable. They may make having vaginal sex painful. They may also make it difficult if you need to have internal examinations in the future.

Your doctor, radiographers or nurse can give you advice about preventing or coping with vaginal changes. Try to follow their advice and ask for more help if you need to. It can be hard to talk about these side effects, but your radiotherapy team can usually help.

We have more information about [coping with vaginal dryness and preventing narrowing of the vagina](#) when having pelvic radiotherapy.

Vaginal discharge



You may have a small amount of vaginal discharge after treatment has finished. It should not be much, and it should only last a few days. Talk to your specialist nurse or cancer doctor if:

- you are worried about this
 - it continues
 - it becomes heavy
 - it smells bad.
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Vaginal bleeding



After pelvic radiotherapy, the blood vessels in the lining of the vagina can become fragile. This means they can bleed more easily after sex or after using the vaginal dilators. Bleeding may be caused by:

- vaginal tissue sticking together
- scar tissue causing the vagina to narrow.

If you have any bleeding, always tell your cancer doctor or nurse. They will examine you and explain whether it is a side effect of the radiotherapy.

Pubic hair

When you have pelvic radiotherapy, you may lose some of your pubic hair. After treatment, it usually grows back but may be thinner than it was before.

Skin reactions

It is not common to have a skin reaction with pelvic radiotherapy, but rarely the skin in the area that is treated may:

- if you have white skin, your skin may redden
- if you have black or brown skin, your skin may darken
- feel sore or itchy.

Your radiographer or specialist nurse will give you advice on taking care of your skin. If your skin becomes sore, itchy or changes colour, tell them straight away. They can give you advice and treatments if needed.

Skin reactions should get better within 4 to 6 weeks of treatment ending.

Possible late effects of pelvic radiotherapy

Most people have side effects during pelvic radiotherapy. Usually these gradually improve over a few weeks or months after treatment finishes. [Late effects](#) may affect you during or shortly after treatment and last longer than 3 months. Late effects may begin months or even years later as a delayed response to treatment. Common late effects include:

- changes to how the bladder or bowel work
- changes to your sex life
- tiredness.

Late effects might be mild and not affect your daily life, but sometimes they can be more difficult to live with. If you have late effects, there are usually things that can help you to cope and live life as fully as possible. It is important that you do not feel alone. Help and support is available. A few hospitals have special clinics for people with late effects.

It is always important to tell your GP or cancer doctor about any new symptoms that develop a long time after treatment. They need to be checked, as they may not be caused by radiotherapy.

Internal radiotherapy (brachytherapy)

Internal radiotherapy is also called brachytherapy. It gives a high dose of radiotherapy directly to the [top of the vagina, or to the vagina and womb](#). This depends on whether you had [surgery](#) to remove your womb. It also gives a high dose to the area close by. You may have brachytherapy on its own. Or you may have it at the end of your external radiotherapy treatment. You usually have this as an outpatient.

Having internal radiotherapy (brachytherapy)

There are 2 different ways to give brachytherapy depending on whether you have had a [hysterectomy](#). Your radiotherapy team can explain what type of brachytherapy is suitable for you and how many sessions you will have.

The brachytherapy is given through the plastic applicator. The machine is operated by a radiographer or a medical physics technician. The machine places a radioactive capsule (source) into the applicator until you have had the planned dose of radiation. When the treatment is finished, the radioactive source goes back into the machine. You are not radioactive after having treatment.

Vaginal brachytherapy



When you go for your treatment session, the plastic applicator is put into your vagina. The end of the applicator that is outside your body is attached to the bed and connected to the brachytherapy machine. The radiographer will then leave the room, but they can see you and talk to you through an intercom.

After treatment, the nurses will then take out the applicator and you can go home.

Brachytherapy to the vagina and womb



If you still have a womb, you may have brachytherapy using applicators into the womb as well as the vagina.

When these are put in, you will have either a:

- general anaesthetic – this means you are asleep
- spinal anaesthetic – this means you are numb from the waist down and unable to feel anything.

If you have a spinal anaesthetic, you will be given some sedation to make you feel relaxed and sleepy. You will also have a small, plastic tube put into your bladder (catheter) to keep your bladder empty.

Once the applicators are in place you will have an [MRI scan](#) or [CT scan](#) to check the position. Later the same day, you will go to the radiotherapy department for your first treatment. The applicators and catheter are removed after your treatment before you go home.

Side effects of brachytherapy

External beam radiotherapy causes more side effects than internal radiotherapy. If you are having brachytherapy only, it is not common to get side effects after treatment. The treatment only affects a very small area. This means it is not likely to cause any long-term side effects, for example to your bladder or bowel. But you may notice some effects to the vagina and your sex life after treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our womb cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.



Womb cancer forum

An anonymous network of people affected by womb cancer which is free to join. Share experiences, ask questions and talk to people who understand.

Get help